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QUALITY MANAGEMENT SYSTEM

STANDARD OPERATING PROCEDURE

Medical Device Vigilance and Adverse Event Reporting

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Applicable Regulatory Standards	EU MDR 2017/745 Articles 87 - 92 FDA 21 CFR Part 803 (MDR) ISO 13485:2016 Clause 8.2.3

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1.0 Purpose

This Standard Operating Procedure (SOP) defines the operational workflow and legal thresholds for notifying external national regulatory competent authorities, ministries of health, and notified bodies regarding serious incidents, field malfunctions, and safety-related corrective actions. This program ensures statutory milestones are fulfilled within absolute legal timelines to preserve public safety and institutional compliance.

2.0 Scope

This procedure dictates mandatory reporting assessment rules for all serious incidents and field safety corrective actions (FSCA) involving devices commercialized globally. It applies from the moment an incident is triaged as a potential reportable event until the official closure of the administrative file with the respective inspectorate.

3.0 Responsibility and Authority

3.1 Head of Regulatory Affairs (RA)

Holds final accountability and corporate authority to sign off on vigilance reports, authorize international submissions, and serve as the main liaison with regulatory bodies such as the FDA, European National Competent Authorities (NCAs), and Notified Bodies.

3.2 Vigilance Assessment Panel

An ad-hoc regulatory assessment panel convened by RA, incorporating Quality Engineering, Chief Medical Officers (or clinical consultants), and Design Authority Leads to determine root cause severity and systemic user risks.

4.0 Procedure

4.1 Screening Criteria for Reportable Incidents

The Regulatory Affairs team must screen all escalated post-market files against a three-part cumulative test to identify legal reportability. An event is reportable if it meets all three criteria:

1. An event or malfunction has occurred involving the medical device (including flaws in labeling or instructions for use).
2. The device is suspect as a direct or indirect causal factor or contributor to the incident scenario.
3. The event led, or might have led, to death or a serious deterioration in the state of health of a patient, user, or other person.

4.2 Strict Reporting Timelines Matrix

Once an event is confirmed as reportable, the RA Specialist must transmit formal notifications via electronic portals (e.g., FDA eSubmitter, European EUDAMED/MIR) according to strict statutory deadlines:

Incident Type Profile	Regulatory Threshold Definitions	Max Deadline Timeline
Public Health Threat (Serious Risk)	Any incident that poses an immediate risk of death, serious injury, or widespread illness.	Within 2 Calendar Days
Death or Unanticipated Serious Deterioration	An incident where a patient/user died or suffered permanent or life-altering impairment.	Within 10 Calendar Days
Standard Serious Incident / Malfunction	A device malfunction or deterioration in performance that could lead to death or serious injury if it recurred.	Within 15 Calendar Days
FDA 21 CFR 803 Baseline Malfunction	Standard US FDA Medical Device Report (MDR) for market malfunctions or injuries.	Within 30 Calendar Days

CRITICAL COMPLIANCE NOTICE: If uncertainty exists regarding whether an incident meets reporting thresholds, regulatory guidance mandates that a report must be submitted within the required timeframe. Delaying a submission to wait for a complete technical root-cause investigation is a clear compliance violation.

4.3 Formulating and Submitting Manufacturer Incident Reports (MIR)

Reports must be compiled using standardized electronic templates (e.g., the XML-driven European Manufacturer Incident Report form). The report must include accurate device identifiers (UDI-DI, Basic UDI-DI), manufacturing lot sequences, clinical event details, and initial corrective actions. Follow-up and final reports must be submitted as additional laboratory engineering findings become available.

4.4 Field Safety Corrective Actions (FSCA) and Notices (FSN)

If a technical or manufacturing issue requires removing a product from the market or executing a field modification (such as software patches or labeling updates), the RA team must draft a Field Safety Notice (FSN). This notice must be submitted to all affected National Competent Authorities for review before it is distributed to clinical end-users.

5.0 Documentation Archiving and Audit Readiness

All vigilance assessment logs, cross-functional panel minutes, electronic submission confirmations, and formal regulatory closure letters constitute critical regulatory records. They must be archived securely for a minimum of fifteen (15) years from the submission date, ensuring they are readily accessible during corporate inspections.

6.0 Associated Reference Materials

- **SOP-QA-003:** Complaint Handling and Post-Market Intake
- **SOP-RA-004:** Post-Market Surveillance (PMS) Program

- **SOP-QA-013:** Product Recall and Field Actions Program

7.0 Revision History Log

Revision Level	Effective Date	Author / Originator	Detailed Description of Modifications
00	[Date]	[Name], RA	Initial baseline release of formalized Medical Device Vigilance and Adverse Event Reporting Standard Operating Procedure.